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ETHIOPIAN PHARMACOLEGAL HISTORY AND CURRENT LANDSCAPE

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Historical Development of Ethiopian Pharmacolegal Systems

Early & Traditional Context

- Pre-20th century: Dominance of traditional and herbal medicine; community-based healing practices.
- Early 20th century: Introduction of modern drugs and rudimentary regulation under imperial administration.
- 1960s–1990s: Initial formal legal instruments (e.g., older regulations such as Regulation 288/1964) provided early legal basis for medicine control.

Institutionalization of Drug Regulation

- Until 1999: Pharmacy & Laboratory Department within Ministry of Health handled medicines.
- 1999 Proclamation No.176/1999 led to establishment of Drug Administration and Control Authority (DACA) — institutionalizing regulation.
- 2010: DACA restructured into Food, Medicine and Health Care Administration and Control Authority (EFMHACA) under Proclamation No.661/2009.

Modernization & Current Framework

- 2019–present: Regulatory modernization under Proclamation No.1112/2019 establishing the Ethiopian Food and Drug Authority (EFDA).
- Focus areas: GMP implementation, combating substandard/falsified medicines, strengthened licensing and inspection.
- Public health research institutions (formerly National Research Institute of Health) merged into EPHI in 1995—supporting surveillance and policy.

Ongoing Challenges & Lessons

- Implementation gaps: need for regulatory tools, human resources, and infrastructure to fully operationalize laws.
- Integration of traditional medicine, pharmacovigilance, and private sector compliance remain priorities.
- Lessons: Progressive legislation has strengthened legal basis, but sustained capacity-building is critical.

Current Regulatory Framework and Institutions

The Pharmacists and Druggists Proclamation No. 43/1942

- In Ethiopia, even though “The Pharmacists and Druggists Proclamation No 43/1942” was used to regulate both the professions and the facilities where they were practiced, comprehensive regulation of the pharmaceutical market was introduced in 1964 by a regulation called “Pharmacy Regulation No. 288/ 1964”.

Pharmacy Regulation No. 288/1964

Comprehensive regulation of the pharmaceutical market was introduced in 1964 by a regulation called “Pharmacy Regulation No. 288/ 1964”. This legislation formed the legal basis for official establishment of drug regulation in the history of Ethiopia, enabling the regulation of the practice of pharmacists, druggists and pharmacy technicians; manufacturing, distribution, and sale of medicines.

Drug Administration and Control Proclamation No. 176/1999

- In June 1999, a new regulation called the “Drug Administration and Control Proclamation No. 176/1999” repealed most parts of the regulation 288/1964. The law established an independent Drug Administration and Control Authority (DACA) with further mandate of setting standards of competence for licensing institutions/facilities.

Proclamation No. 661/2009

DACA was re-structured as Food, Medicine and Health Care Administration and Control Authority (EFMHACA) of Ethiopia by the “Proclamation No. 661/2009” in 2010 bearing additional responsibilities like regulation of food, health care personnel and settings.

Institutions

The Ethiopian pharmacological system is regulated and supported by several key institutions at the federal level. The two most prominent and central bodies are:

1. Ethiopian food and drug authority (EFDA)

- This is the National Regulatory Body for all pharmaceutical and health products. Its core mandate in the pharmacological system is to safeguard public health by ensuring the safety, quality, and efficacy of medicines.

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Its key roles includes:

- Licensing of pharmaceutical manufacturers, wholesalers, exporters, and importers.
- Assessing and registering medicines before they can be legally imported, manufactured, or sold.
- Conducting inspections of facilities and monitoring the market for substandard, falsified, or illegal products.
- Monitoring the safety of medicines after they have been approved and marketed (Adverse Drug Reaction reporting).
- Issuing and revising the National Essential Medicine List (EML) and standard treatment guidelines.

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2. Ethiopian Pharmaceuticals Supply Service (EPSS)

- Formerly known as the Ethiopian Pharmaceuticals Fund and Supply Agency - PFSA
- This institution is primarily responsible for the pharmaceutical supply chain for government health facilities.

Key roles includes:

- Procuring and purchasing essential medicines and medical supplies for the public sector.
- Managing the warehousing and distribution of pharmaceuticals to public health facilities (hospitals, health centers) across the country.
- Implementing and managing the Integrated Pharmaceuticals Logistics System (IPLS).

Other Related Institutions

- **Ministry of Health (MOH):** The overarching body that provides policy direction, strategy, & supervision for the entire health sector, including the pharmacological system.
- **Higher Education Institutions (HEIs):** Such as Schools of Pharmacy at various universities, which are responsible for training pharmacists, pharmacologists, and other pharmaceutical professionals.
- **The Health and Health-related Institutions and Professionals Regulatory Lead Executive Office (HHrIPR-LEO),** under the MoH, is responsible for the Ethiopian Health Professionals Licensing Examination (EHPLE).

Core Challenges in the Pharmaco-legal System

- Legislative Fragmentation: Outdated, dispersed laws; gaps in modern regulation.
- Institutional & Capacity Limitations: Lack of technical staff, labs, enforcement systems.
- Weak Lifecycle Oversight: Focus on pre-market only; limited pharmacovigilance; informal medicine trade.
- Operational Inefficiency: Manual, paper-based processes; delays; lack of transparency.
- Low Awareness: Poor public knowledge on rational medicine use; weak ADR reporting; limited industry guidance.

Major Reforms & Strategic Initiatives

- Legal Reform: Proclamation 1112/2019 → EFDA autonomy.
- Digital Transformation: EFDA e-Service Portal (Mekellebank); streamlined licensing, registration, GMP certification.
- Scientific Capacity Building: Upgraded National Quality Control Laboratory; expanded pharmacovigilance nationwide.
- Harmonization & Guidance: New guidelines for clinical trials, medical devices; alignment with regional/international standards.

Future Outlook & Strategic Direction

- Vision: Best-in-class African regulatory authority.
- International Benchmarking: WHO Maturity Level 3 → WHO Listed Authority (WLA).
- Regional Integration: Active role in African Medicines Agency (AMA); harmonization and mutual recognition.
- Advanced Regulation: Integrated digital ecosystem; risk-based inspections.

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- Health Security: Support local pharmaceutical industry; strengthen national health security.
- Public Vigilance: Continuous surveillance, pharmacovigilance, and education campaigns.
- Conclusion: Ethiopia's pharmaco-legal system has transformed; future aims at global recognition, regional leadership, and strong public health protection

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